

# Pain relief and anaesthetic information for women taking blood thinning injections during pregnancy

Information for patients

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## **Why have I received this leaflet?**

The obstetric anaesthetic team have been informed that you are taking a blood thinning injection either called enoxaparin or dalteparin. This is important because some of the choices for pain relief and anaesthesia when you come in to have your baby depend on **the timing of your last dose** of blood thinning injection.

The injections aim to thin your blood a little to reduce the chance of you developing blood clots in your legs or lungs. We have to be careful with the timing of some types of pain relief and anaesthetics which are given through small needles in your back. If they are given too soon after the blood thinning injection, they can cause a bruise which can press on nerves in your back which can affect your legs.

## **What are the types of pain relief and anaesthetics which are given through needles in your back?**

An epidural is frequently used for relieving pain during labour. A thin tube ("catheter") is threaded through a needle in your back and local anaesthetic drugs are given through the tube for pain relief. An epidural can also be used as a type of anaesthetic if you need to have an operation (e.g. a caesarean section or forceps delivery). In this case stronger drugs are given down the epidural tube to make your legs and stomach very numb.

A spinal anaesthetic is often used to provide anaesthesia for an operation if you don't have an epidural (e.g. a caesarean section or forceps delivery) in theatre. Local anaesthetic drugs are given through a spinal needle in your back as a one-off injection.

## **What happens if it is more than 12 hours from my last dose of blood thinning injection and I need pain relief in labour?**

If it is more than 12 hours from the last dose of blood thinning injection, we will treat you "as normal": all options are open for pain relief in labour including "gas and air" (Entonox), morphine and epidurals.

## **What happens if it is more than 12 hours from my last dose of blood thinning injection and I have to have an anaesthetic for my baby to be born in the operating theatre?**

If your baby needs to be born in the operating theatre by forceps or caesarean section all options are open to you for anaesthesia. We recommend that most women are awake to have their baby with an epidural or spinal anaesthetic: your anaesthetist will chat to you before your anaesthetic and will discuss which options are safest for you and your baby.

## **What happens if it is less than 12 hours from my last dose of blood thinning injection and I need pain relief in labour?**

If it is under 12 hours from the blood thinning injection it is not safe to put a needle in your back and therefore an epidural is not possible. If you want to have strong pain relief, we can offer you a remifentanyl PCA (patient controlled analgesia) which is a good alternative to an epidural for pain relief.

The options for pain relief are “gas and air” (Entonox), morphine or a Remifentanyl PCA. We need to wait 4 hours from a dose of morphine before we can give you Remifentanyl PCA safely. It is therefore better to try the Remifentanyl PCA before morphine. Enclosed is a leaflet explaining more about Remifentanyl PCA. You can safely use “gas and air” (Entonox) and Remifentanyl together.

## **What happens if it is less than 12 hours from my last dose of blood thinning injection and I need to have an anaesthetic for my baby to be born in the operating theatre?**

If it is under 12 hours from the blood thinning injection it is not safe to give you an epidural or spinal anaesthetic into your back. In this situation we would plan for you to have a general anaesthetic- which is an asleep anaesthetic.

A general anaesthetic involves a special drink as the first step to neutralise the acid in your stomach. Routine monitors are attached including a sticker onto your forehead to monitor you closely when you are asleep. Then you are given some oxygen to breathe and our assistant has a very gentle feel of the front of your neck. This is to help stop any food or acid coming up into your mouth as you go off to sleep.

We ask your birth partner to wait in the labour ward as we put you off to sleep so the theatre team can fully concentrate on you. Your baby will receive a small amount of the anaesthetic too, but will go straight to the baby doctors in the theatre suite and soon will be awake.

At the end of the operation you will be woken up in the theatre and we will transfer you to the recovery area where your partner and baby will be able to join you. It is important your birth partner takes lots of photos for you, to help record these first moments together for you.

For more information, please scan the following code or use the web address below:

[https://www.labourpains.org/assets/\\_managed/cms/files/New%20Leaflets/Labour-Pains-Caesarean-Section-Information-sheet-EN2.pdf](https://www.labourpains.org/assets/_managed/cms/files/New%20Leaflets/Labour-Pains-Caesarean-Section-Information-sheet-EN2.pdf)

## **What should I do about taking my injection of blood thinning injection if I think I might be going into labour?**

If you think you might be going into labour don't take your injection but ring Obstetric Triage and discuss your symptoms with the Triage Midwives.

Triage at the Royal Infirmary of Edinburgh (RIE): **0131 242 2657**

## **Where can I get more information?**

Please contact the Obstetric Anaesthetic Secretary on **0131 242 3151** if any of this information is not clear and a telephone call can be arranged with an Anaesthetic Consultant.

For more information on Obstetric Anaesthesia please visit: [www.labourpains.org](http://www.labourpains.org)